

Product Sophistication: Where Each Vendor Sits

Six components of sophistication, each on a five-step scale. Positions are our read of the demo calls and each vendor's RFP return. Vendors in alphabetical order.

Compassus Home Health
Internal, for discussion

AR Arya **AX** Axle Health **CS** CareStitch **VC** VitalisCare **----** Compassus Assist boundary for assignment and coverage (acts, with approval)

Decision-making Who makes the call	1 · Shows data	2 · Filters who is eligible CS	3 · Recommends AX VC	4 · Acts, with approval	5 · Acts on its own AR
Automation Who does the work	1 · Person does it, better tools	2 · System prepares, person does CS	3 · System does, person approves each AX VC	4 · System does routine; people get exceptions AR	5 · End to end, exceptions too
Engagement Patients and clinicians	1 · None VC	2 · Person-led, with tools CS	3 · One-way automated notices AX	4 · Two-way automated text AR	5 · Two-way text and voice agent
Data visuals What leaders and schedulers see	1 · Lists	2 · Calendars and maps AR	3 · Capacity by clinician and day CS	4 · Forecasts and leader dashboards AX VC	5 · Predictive, with reasons
Learning How it gets better	1 · Fixed rules AX	2 · Logs feedback for people CS	3 · Override reasons retune ranking VC	4 · Learns rules from feedback AR	5 · Learns, and shows what it learned
Fit: enterprise and branch How it is shaped to Compassus	1 · One setup for all	2 · Enterprise settings	3 · Enterprise plus branch settings AX VC	4 · Branch rules, weights and autonomy AR CS	5 · Branch and group, self-serve, governed

What puts each vendor where it is

	Arya	Axle Health	CareStitch	VitalisCare
Decision-making	Scorecard: Runs it. Decides and acts with no per-event approval on outreach and call-outs (demo). New-case staffing had an approval gate for the demo; normally the office is told, not asked (1:25).	Scorecard: Recommends it. Clinician, day and time with every factor (demo). Can run alone; every customer keeps a person (RFP).	Scorecard: Checks it. Filters and shows; the scheduler decides (demo). Recommendation engine, then agentic AI, on roadmap.	Scorecard: Recommends it. Top four per discipline, better-day flags; scheduler approves (demo, RFP).
Automation	Texts patients and clinicians, re-staffs call-outs, writes to HCHB; office sees exceptions (demo).	Bulk-plots the episode, call-out flow in seconds; person approves (demo). Write-back switched off.	Staffing requests go out to priority groups automatically; each assignment is manual (demo, RFP).	RPA writes approved assignments to HCHB (demo). Call-Off Center triage in development (RFP).
Engagement	Two-way text live: discharge check, preferences, arrival time (demo). Voice and Spanish claimed.	Arrival-window text; replies go to the clinician (demo). RFP: voice, text, email, person in loop advised.	Click-to-call with a call log (demo). Text confirmation on roadmap (RFP).	No patient messaging, no date (demo, RFP). Clinician app for routing.
Data visuals	Worklist, route map, agent-health insights (demo). Rich maps and calendars deprioritized by design (RFP).	Capacity forecast by zip, utilization and productivity analytics (demo).	Points grid by clinician and day, territory polygons, maps (demo). No branch total.	Live map, route-adherence review, predictive benchmark (demo).
Learning	A typed rejection becomes a rule or weighting; rejection reports (demo). Rule view being built.	Logic set from a survey of our rules (demo). Learning from overrides not described.	Replies and reads are logged as data (demo). No model learns from them.	Call: override reason trains the model. RFP: clinician disagreement does not retrain it.
Fit: enterprise and branch	Org rules inherited and overridden per branch; we set hard vs soft rules; autonomy by branch, visit type (demo, RFP).	Our logic captured by survey; permissions and accept/decline rules by role (demo, RFP).	Self-service properties, priority groups, territories, sub-organizations (demo). Governance is on us.	Rules per branch and discipline, then per person (demo).

Runs without anyone approving each time

Arya	Patient discharge check and preference texts; clinician onboarding texts; outreach to eligible clinicians, best fit first; a call-out re-formed across the week, office told only if it can't; learning rules from rejections (demo). Visits released when auth clears; capacity windows re-checked continuously (RFP). New-case staffing: office told, not asked, once past co-pilot (call).
Axle Health	Work queues built automatically; patient texts when the clinician locks a time or heads out (demo, RFP). Assignments, dates and coverage: a person approves every time (RFP).
CareStitch	Staffing requests sent to priority groups with delays and reminders; a care-team chat per patient (demo). Optional auto-assign to the first clinician who says yes (demo). Other assignments are manual.
VitalisCare	The clinician's route re-calculated as the day changes (demo). Assignments are written to HCHB only after the scheduler approves (demo, RFP).

How to read it

- **Further right is more capable, not automatically a better fit.** Our Assist boundary for assignment and coverage is "acts, with approval"; past it, we need approval kept switched on.
- **Set-up is not approval.** Only a person approving each decision counts.
- **Learning and branch fit** need an owner at Compassus to review rules across 80+ branches.
- Decision-making mirrors the formal scorecard mark; the other rows are our read.

Sources: demo calls on 22, 23, 24 and 28 Sep 2026, and the vendors' RFP returns (scorecard workbook 9.11.26). "Demo" means seen on the call; "RFP" means described in the return, not demonstrated. Claims not shown on a call are placed where the vendor says they sit and flagged in the evidence. This page compares; it does not rank or recommend.